



Government of **Western Australia**
East Metropolitan Health Service
Armada Kalamunda Group

Armada Kalamunda Group
East Metropolitan Health Service



OFFICIAL

Adult Community Mental Health

Clinical Treatment Team

Process Manual

Authorisation

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About this Manual

This manual provides a clear framework of procedures and processes for mental health clinicians to follow who are employed in the Clinical Treatment Team (CTT) for Armadale Adult Community Mental Health Service (ACMHS) located at Orchard Avenue Centre Armadale.

Model of Care

The East metropolitan Health Service (EMHS), ACMHS, CTT model of service describes the function of one component of a broader mental health system.

CTT provides medium to long term recovery focussed, specialist community mental health care services for individuals living in the Armadale Health Service (AHS) catchment areas.

This document should be read in conjunction with the EMHS [Care Coordination in Mental Health Policy](#)

Principles and Key Functions

This service is provided with a recovery orientated approach that is person centred, strengths based, collaborative and empowering. The lived experience and insights of people with mental health issues and their families are at the heart of recovery orientated culture. The service meets the National Standards (NSQHS) for mental health services and the National Practice standards for the mental health workforce and is consistently audited and reviewed alongside the broader mental health systems. CTT operates on the premise that people can and do recover from mental illness.

The EMHS and Armadale Kalamunda Group (AKG) Mental Health Adult Program principles are:

- Based on EMHS and AKG Vision and Values
- No wrong door
- Easy Access
- Responsive to referrals
- Reducing risk to consumers, their family, carers and staff
- Timely mental health intervention.

The **Key Functions** are:

1. Provision of assertive clinical management to minimise the impact of mental illness on consumers and their family and/or nominated personal support persons. (PSP)
2. Facilitation of access to a broad range of clinical and non-clinical services to assist recovery.
3. Working in partnership with consumers to develop their personal support systems and to assist them to engage in their communities.
4. Engagement with non-government organisations, primary care, and other service providers to assist access to services across the recovery continuum and ensure smooth transitions of care.
5. Facilitate, encourage, and support consumers to direct their own recovery journey.
6. Supporting families and nominated PSP to access any appropriate support.
7. Increase the level of mental health literacy and understanding to promote social inclusion.

Access

All adult community referrals to the ACMHS are via the Assessment and Treatment Team (ATT). This facilitates ease of access with a clear and consistent process for new referrals, and re-referrals from other services. Referrals for outpatient services from the Armadale Kalamunda Group (AKG) Adult MH inpatient unit and Crisis Resolution Home Treatment Team (CRHTT) can be directly referred to CTT.

Entry criterion for CTT referrals:

- Persons (aged 18 - 65 years) residing in the Armadale Mental Health Service catchment area.
- Persons who require specialist mental health intervention and support for longer than the nominated length of stay within the ATT program.
- Persons with significant and complex mental illness and co-occurring functional difficulties that require coordination of care and support across multiple agencies/providers, family and/or nominated PSP's
- Persons with significant and complex mental health issues who are not responding to treatment prescribed by their current care provider.
- Persons already engaged with a CTT transferring between catchment areas where it is identified there is a clinical need for care exceeding that which can be provided in the ATT program.
- Persons whose long-acting injectable (LAI) administration cannot be managed within a primary care setting after alternative pathways for administration have been explored.
- Persons requiring treatment under a Community Treatment Order (CTO) of the Mental Health Act WA 2014
- Persons prescribed Clozapine therapy.

CTT is not suitable for/Exclusion Criterion:

- Referrals purely for attention deficit disorder/attention deficit hyperactivity disorder - redirect to private psychiatrist who specialises in ADD/ADHD assessment and treatment. Consumers with a comorbid mental disorder who otherwise meet the entry criteria will be considered.
- Intellectual Disability - redirect to the GP who can refer to a Neurosciences Unit or a private neuropsychology service.
- Autism Spectrum Disorder – redirect back to the GP who can refer to the Autism Association Diagnostic Clinic, or to a private psychiatrist/psychologist who specialises in ASD assessment.
- Continued prescription of a Schedule 4R/8 drug or medicinal cannabis, stimulants – directed back to the GP with a recommendation to involve an AOD service if required.
- Medical assessment for a Disability Support Pension application - direct back to the patient's General Practitioner
- Complete a driver's license assessment - direct back to the consumer's General Practitioner
- Psychiatric report to evidence a workers' compensation claim, fitness to work or court proceedings – redirect to private psychiatrist specialising in providing such reports.
- Psychological treatment with no indication of requiring multidisciplinary team input, including medical review – redirect to patient's General Practitioner suggesting a Mental Health Care Plan is completed and referral to a private psychologist or suitable community organisation.
- To gain access to medical detox or a rehabilitation service for treatment of addiction - direct to an AOD service
- Where the primary reason for the referral is to undertake a NDIS application - redirect to consumer's general practitioner.
- Where administration of LAI can be managed within primary care setting.

- Individuals with no fixed address within the AHS catchment area.

Hours of Operation

CTT operates from 0830 to 16:30 hours Monday to Friday. Mental Health Services required outside these hours will be managed through ATT, Mental Health Emergency Line (MHERL) or general hospital Emergency Departments.

Care Coordination

AHS follows the EMHS Care Coordination in Mental Health Services Policy. Please review this document to identify your roles and responsibilities within this Policy.

[EMHS Care Coordination in Mental Health Services](#)

The Care coordinator will be a member of the multidisciplinary team who coordinates and monitors delivery of service to the consumer and their family/PSP.

A multidisciplinary treatment team will include a consultant psychiatrist and other health disciplines to foster a multidisciplinary approach to the consumer's needs and interests. There are both shared tasks and specialist tasks in the coordination of care and these are performed across the MDT membership and within specific disciplines respectively.

Documentation

The MDT members are responsible for the completion and review of the following documentation in accordance with the [AMHS: Assessment and Admission Documentation Requirements Policy](#):

- WebPsois Risk Assessment and Management Plan (RAMP);
- WebPsois Mental Health Assessment Form
- WebPsois Treatment Support and Discharge Plan
- Consent to Release/Obtain Information (AKMR30.2).
- Progress Notes (Integrated Health Record – DMR)
- Kessler 10.
- Care Transfer Summary (SMHMR916).
- LUNSERS
- ASSIST
- Metabolic Screening
- PSOLIS Activation/Deactivation Form

The CTT will complete the following upon allocation of the care coordinator.

Referral and Assessment

New referrals meeting the access criteria are made via ATT, the inpatient unit or from other catchments. New referrals and those from other catchments will be triaged by the Triage Officer and assessed by ATT to determine if any immediate interventions are required.

For current CTT consumers, notification of discharge from Inpatient/CHRTT Care is received directly to the care coordinator or First on call (FOC) via IPU ward rounds, verbal/written handover to the identified Care Coordinator or email to the FOC inbox AHS.CTTFOC@health.wa.gov.au

- CTT will establish contact with the consumer as soon as practically possible with, face to face contact no later than 10 working days post acceptance of referral.
- Home based interventions should be prioritised over clinic-based interventions to facilitate involvement of the consumers personal support persons.
- CTT have a process in place with ATT, to accept responsibility for the care of the current CTT consumers referred for afterhours care until the next business day. (see after hours referrals)
- All consumers will have a documented Care Transfer Summary (CTS) if currently receiving care through another CTT service, outlining the areas of unmet need. CTT will provide a progressive, comprehensive, ongoing assessment with each consumer.
- Consumers referred to CTT are activated at the point of acceptance to the team.

Transferring Active ATT Consumers to CTT

This section should be read in conjunction with the WA Health MP 0095 [WA Clinical handover Policy](#) for clinical hand over requirements

Active consumers in the ATT program can be assessed and considered appropriate for the CTT at any stage of the referral or within the ATT episode of care. Transfer of care must be discussed with the consumer/carer and other relevant agencies and occur in a timely manner.

If an active ATT consumer is considered suitable for the CTT following discussion at the ATT team meeting, the following process must be followed:

- The ATT care coordinator will contact the FOC to inform them of the referral.
- Following contact, the FOC will add the referral information to the CTT referral spread sheet (CTT Referrals.xls), located at: W:\Mental Health\AKH\Mental Health\General\CTT Clinical Treatment Team
- Once Statewide Standardised Clinical Documentation (SSCD) is completed, The ATT care coordinator will email the FOC Email address: AHS.CTTFOC@health.wa.gov.au
- The CTT care coordinator, after allocation, will contact the ATT care coordinator and arrange a face-to-face handover if more information is required. Transfer of care is finalised once the handover has taken place after which the consumer will be deactivated from ATT.
- A joint meeting with the consumer, ATT clinician and new CTT care coordinator may be arranged for the purpose of introduction and smooth transition of care if beneficial for the consumer.

- For required transfer documentation see Documents section.
- CTT will complete the Psolis Activation form.

Referrals from an Armadale Kalamunda Group, Mental Health Service Inpatient Ward

Post discharge care pathways will be discussed and planned prior to discharge from an AMHS inpatient ward. This will include identification of the most appropriate community team for follow up, allocation of a care coordinator and participation in discharge planning.

Requests from the inpatient ward for care coordination are made via the daily Stand Up or ward round meetings and confirmed by sending email to the FOC email address.

In Patient staff will be responsible for ensuring there is an up-to-date MHAF, RAMP and CTS available and that a current Intramuscular Long-Acting Injectable Medication Chart (MR58) is forwarded to the Team if applicable.

Discharge from hospital and transition to community-based care is a critical point as consumers may be more vulnerable and at increased risk of readmission or suicide.

People who are discharged from hospital should have contact within 7 days of discharge.

- [Discharge from the Mental Health Inpatient wards](#)
- [EMHS Rate of Community Follow up within 7 days of discharge](#)

CTO Transfer

To ensure the timely allocation and continuation of treatment for consumers discharged to CTT on CTO the following process will occur:

- All requests for CTO from local and out of area will be forwarded to the Team Leader or delegate who will obtain provisional discharge summary, care transfer and CTO referral form.
- The Team Leader or delegate determines which consultant per FTE capacity will be allocated the CTO, notifying all consultants and referring practitioner by email to enable verbal clinical handover between medical staff.
- The accepting Consultant will acknowledge referral to the service by sending an email to the ATT referral email and FOC. The email should include demographics so that the consumer can be identified on PSOLIS, any information that has been gathered regarding the consumer's presentation and
- Confirmation that the CTO has been accepted. ATTReferral.Armadale@health.wa.gov.au or search on Outlook global email as Armadale ATT Referral
- ATT Triage will accept and manage the documentation from the referring services when it arrives. They will also make every effort to contact the consumer or NOK to ensure that the consumer is living at the address provided and is aware of referral to ACMHS.

- A care coordinator will be allocated at the next planned allocation meeting. The consultant psychiatrists will update the spreadsheet when a CTO is revoked.
- On revocation of a CTO, the consultant will inform the Team Leader/delegate who will update the appropriate CTT spread sheet on W drive.

Assessment

All assessments require the completion of the SSCD documentation forms with as much information as possible covering the following domains:

- The reason for referral
- A current mental status examination (MSE)
- Psychiatric history and diagnosis, including onset, course, effects of illness.
- Past treatment
- Efficacy of treatment
- Mental Health Act (MHA) status
- Risk and Risk Management Planning (RAMP)
- Physical health assessment
- Physical appearance form
- Education and employment history
- Social development and functioning
- Cultural assessment
- Activities of daily living
- Family/carers/significant others/nominated persons and relationships.
- Formulation and initial management plan
- Collection of outcome measures (LSP16, NOCC, Kessler 10+)
- Information from the consumer and collateral information from any other service providers, family/significant others

The Care Coordinator and treating doctor are responsible for completing the comprehensive assessment within 10 working days of referral if not already completed.

The Mental Health Assessment Form (MHAF) is to be reviewed and updated annually or if there is a significant change in presentation requiring in patient admission.

Risk Assessment

Risk assessment is currently completed on WebPSOLIS - Risk Assessment Management Plan (RAMP). This is automatically transferred into DMR. These are to be completed on initial review, every three months; or where change in acuity is noted, transfer of consumer, and at discharge.

All risk assessments will assess:

- a) risk of harm to self-including suicide
- b) risk of harm to others
- c) vulnerability risks
- d) any protective or safety factors

All risk issues identified will require a risk plan collaborating with key service providers, family/carers/significant others/nominated persons, and will be actively engaged in the risk assessment process. (With consumer's consent)

Care coordinators will ensure good liaison with any other agencies regarding changes in risk presentation.

Sexual Safety

Everyone accessing mental health services has the right to feel and be psychologically and physically safe. This includes being free of, and feeling safe from, behaviour of a sexual nature that is unwanted, or makes a person feel uncomfortable, afraid or unsafe.

It is important to note that some consumers accessing our service may be more vulnerable.

- Consumers with a cognitive impairment (including cognitive impairment for mental illness) are vulnerable to abuse.
- Individuals who have a history of physical and sexual abuse can have a higher degree of lifetime exposure to sexual abuse and violence.
- LGBTQI+ consumers experience increased vulnerability to sexual victimisation.

As part of a comprehensive assessment, consumers are asked how they identify in relation to gender, ensuring gender sensitive approaches and staff gender preference. Clinician allocation will consider the consumers preferences alongside clinical need and staff availability.

Sexual Safety of Consumers of Mental Health Services

Physical Health Assessment

Physical and oral health will be routinely assessed and documented. This may be conducted by a health service provider external to the CTT but will be considered as part of CTT assessment. Physical and oral health issues will be routinely addressed in partnership with all consumers of CTT.

- All consumers will be supported to engage with a designated GP. Where identified barriers exist.
- The CTT treating doctor and Care Coordinators will maintain written communication with the GP throughout care and any changes in treatment communicated.
- The GP will be contacted for collateral and will be provided with results/ongoing care recommendations.

Drug and Alcohol Assessment

Co-occurring mental health and alcohol and other drugs (AOD) needs are highly

prevalent in the CTT cohort of consumers. Assessments of alcohol and drug use should be undertaken on entry to CTT using the ASSIST form. CTT is expected to complete these assessments unless already undertaken and reviewed whenever there is a change in AOD consumption/use. Management of AOD issues should occur as part of the treatment and clinical review process.

Admission to Inpatient Mental Health Ward/Via ED

At any point in time, inpatient and community teams have a shared responsibility to ensure that inpatient beds are utilised efficiently, so that the most appropriate consumers occupy available inpatient beds.

Assertive admission and early discharge strategies will be utilised concurrently to ensure access to beds for acutely unwell community consumers.

When inpatient units experience high levels of acuity, referring community clinicians will explore alternative options. If the consumer is awaiting a bed due to deterioration in their mental state, they will be contacted daily for continued monitoring/intervention until a bed becomes available and after-hours ATT utilised for additional support as necessary.

Please refer to:

- [Brag Action Cards](#)

All admissions/presentations will be in accordance with the processes/procedures contained within relevant policies and guidelines including the following:

AMHS

- [*Admission of Patients from Community Mental Health Service to Armadale Mental Health Inpatient Units.*](#)
- [WA Health MP 0095/18 Clinical Handover Policy](#)
- [Discharge from the Mental Health Inpatient Wards and Community Mental Health Teams](#)

DoH

- [Mental Health Bed Access, Capacity and Escalation](#)

EMHS

- [Mental Health EBM Business Rules](#)
- [Mental Health Bed Access, Capacity and Escalation Framework](#)

Where an assessment indicates that admission to an inpatient ward or presentation to the AHS ED may be required, the CTT clinician will liaise with the medical team to confirm the identified plan.

If presentation is to the Emergency Department (ED) then the Consultant from CTT must contact the ED Consultant prior to the consumers attendance.

Documentation is updated as per inpatient admission requirements.

Prior to admission to ED/Inpatient unit, the MHAF and RAMP is to be completed by, the admitting CTT clinician/Medical staff and include, current presentation and formulation inclusive of mental state assessment, current treatment and plan for admission. AKG [Assessment and Admission Documentation Requirements](#).

The admitting clinician will make all reasonable attempts to contact any family members, or PSP's/agencies identified and consented to by the consumer, to inform of the admission. Any outstanding action/contact will be handed over to the ED Liaison Nurse or ward Shift Coordinator for follow up.

In the absence of the CTT care coordinator it is the responsibility of the FOC to coordinate the admission of the consumer.

The consumer's care will be transferred to the admitting ward; however, they will remain active on PSOLIS in the CTT program.

The CTT care coordinator must be involved in all discharge planning and in reach into the ward during their consumers admission unless it is decided that the consumer will not be discharged back to the CTT. This includes being invited to/attending the ward family/discharge planning meetings and contribution to the treatment support discharge plans and clinical reviews.

The inpatient treating team will inform the CTT at the daily morning huddle meeting of all consumers identified for discharge to the CTT. The consumer will be allocated a care coordinator within 24 hours who will commence in reach and active participation in discharge planning.

Documentation:

The following documentation is required for all admissions/ Emergency Department Presentation:

- Mental Health Assessment Form (SMHMR902).
- WebPSOLIS Risk Assessment and Management Plan (RAMP)
- Progress Notes (DMR)

Discharge NOCC measures are to be completed and entered on PSOLIS.

Multiple ED Presentations by Active Consumers of Community Mental Health Service

Active consumers of CTT who present to the Emergency department on more than two occasions in any four-week period irrespective of the reason will have their care reviewed by their care coordinator and treating doctor. (This may be an MDT discussion depending on type of presentation)

The ED Liaison nurse will forward any documentation to the First on Call Email address for the attention of FOC/CC noting the number of times that the consumer has presented in the last four weeks. and requesting a review of the consumer and their TSPD and or Crisis Plan.

Following notification, the Care coordinator or FOC will contact the consumer to review the reasons for presentation, ascertain current mental or physical state and risk, identifying any immediate intervention required notifying the treating doctor of the outcome. The current TSDP/Crisis plans will be reviewed and updated as appropriate.

If an ED presentation is for physical issues, the treating team will follow the above process to review if any psychiatric treatments may be relevant to the presentation.

Clinical Review

Clinical reviews are a formalised meeting with the multi-disciplinary team to review identified management actions and share information to ensure consumers receive ongoing assessment, monitoring, and treatment via a coordinated multi-agency response.

A clinical review will be conducted every 91 days. The reviews are scheduled every week and chaired by the consultant psychiatrist.

- The MDT is responsible for ensuring the clinical quality and standards of care.
- The Team Leader or appropriate delegate will take responsibility for monitoring the compliance of assessments, NOCC data collection, RAMP and TSDP completion.
- The review process will incorporate and reinforce recovery-oriented practice e.g., stages of recovery, consumer choice, strengths, and goals and phase of care.
- All service providers involved will be considered, and where possible included, in care planning, discharge planning and review processes.
- Active cases are discussed with the multidisciplinary team as per scheduled review date or where clinically indicated,
- Clinical reviews are documented in PSOLIS under the Clinical Review Tab
- There is an established clinical review agenda for discussion of cases, a record of attendance and concise documentation of the ongoing plan of care.
- Significant changes in intervention will be discussed at the multidisciplinary team clinical reviews.
- Critical events will be reviewed according to the clinical incident management standard and EMHS, MH processes.

Treatment Support and Discharge Planning

Involvement of family/significant others/nominated PSP's and other service providers is recognised as integral to consumer engagement and is strongly linked to better clinical outcomes.

TSDPs are formulated on entry to CTT and are formally reviewed every three months and updated at a frequency determined by change or need. Plans will be developed in collaboration with all relevant providers, in the presence of the consumer and their identified support network and will:

- Be recovery focused and unique to the individual.

- Detail specific interventions,
- Identify clear treatment and discharge goals in accordance with the clinically significant needs identified from the collection of NOCC data.
- Include the specific issue (including physical) and what is being done by who, when and where.
- Be developed in partnership with the consumer, PSPs and other agencies as defined by the consumer's goals.
- Be printed and offered to the consumer with a signed copy uploaded to DMR.

As the consumer transitions through their recovery stages the level and type of support will be determined by the consumer's goals. Every effort must be made to supply resources to families/ carers/significant others which may be helpful to them.

Discharge and Transfer of Care

Discharge can be a time-intensive aspect of care coordination. The responsibilities of CTT in discharge planning can be summarised into key areas. These areas also overlap with the criteria mentioned in the National Safety and Quality Health Service (NSQHS) Standards [NSQHS](#) and the Chief Psychiatrist's Standards for Clinical Care [OCP clinical Care Standards](#)

- Engage in collaborative discharge planning at an early stage of treatment with the consumer and their family/nominated PSP and key stakeholders.
- Safe transition of care – the nature and length of the transition period will vary according to the history and care needs of the consumer.
- Provide written and verbal treatment-related information for the consumer being discharged to continuation of treatment in an alternative setting.
- Re-entry pathways – If the consumers' needs change post discharge and their existing plan becomes inappropriate to their needs, they have clear advice on where to seek support, treatment, or re-referral to the specialized service.

Discharge Criteria

- Persons no longer resident within the service's catchment area.
- Those who have not received services from CTT for an extended period due to e.g. no engagement/multiple DNA's.
- Those who do not require specialist mental health services. For example, individuals who have been stable for a substantial period and whose psychiatric and psychosocial needs could be provided by a GP, private psychiatrist, or other health care provider.
- Those who require specialist psychiatric services and are compliant with treatment, and whose needs could be provided for by a private psychiatrist or other appropriate service providers.
- Care Transfer Summaries (WebPSOLIS) are to be completed on Discharge/transfer of care to another mental health service. Copies provided to consumer, nominated General Practitioner, carer/nominated PSP and if relevant to non-government organisation/stakeholder, (with consumer consent).

- A comprehensive discharge summary letter is sent to the nominated GP when discharging directly to primary care setting.
- Nocc data collection and RAMP are recorded on Psolis/WebPsolis at point of discharge from service. Closure of TSDP/Crisis Plans
 - [EMHS Outpatient Clinic Discharge Procedures for patient Led Cancellations and Non-Attendance](#)
 - [AKG Scheduling, Rescheduling and DNA of Appointments in Community mental Health Clinics](#)

Disengagement from Service by Consumers:

- To increase engagement, care coordinators should consider sending an Essendex SMS message 24-48 hours prior to the consumers scheduled appointment, particularly if the consumer is likely to DNA.
- Admin will facilitate the SMS for all medical appointments.
- Failure by a consumer to attend a planned appointment with either the care coordinator or the doctor will be considered a valid DNA and recorded in DMR and Psolis
- If the consumer fails to attend two scheduled appointments (either medical or care coordinator), without contacting the clinic, the care coordinator will automatically send out a two-week template letter to the consumer, their next of kin, GP and any other agencies involved in their care.
- There will be an escalation of care to the treating doctor informing them that the two-week letter has been sent.
- When the two-week deadline has passed, if no contact has occurred, the care coordinator will raise with the treating team at the next scheduled clinical meeting.
- The treating team will review and consider all potential risk issues including potential for relapse before making the decision to transfer the care to the primary care provider. The decision to discharge will be with the approval of the consultant psychiatrist.
- If discharge from service is identified, a discharge template letter will be sent to the consumer, next of kin, GP and other agencies detailing the re-entry pathway and out of hours contact numbers.

Clozapine and Depot Clinics

- The Clozapine and Depot clinics are facilitated by a Registered Nurses (RN's).
- Consumers on Clozapine attend the clinic to ensure their mental and physical health is monitored as per the [AKG Clozapine Prescribing, Administering and monitoring Procedure](#)
- Consumers who attend the clinic for administration of depot medication will have their mental and physical health assessed, any identified concerns are to be communicated to the Care Coordinator and treating doctor prior to the medication administration.

- Alternatively, depot administration may be undertaken by the consumers care coordinator outside of the depot clinic times at the clinic or by domiciliary visit.
- Depot medications are administered in line with the [AKG Long Acting Injectable Depot Medication in Community Mental health Clinics Procedure](#)
- The Care Coordinators remains responsible for their consumers attending the clinic and are expected to follow up when consumers do not attend for their scheduled appointments.
- [DoH Clozapine resources](#)

Pharmacological Interventions

Side effect monitoring will be routinely conducted with emphasis on metabolic complications of Psychopharmacological treatment.

- Consumers and their family/nominated persons will receive education and information regarding medication management. Consumer's personal goals for medication will be incorporated with staff's clinical knowledge.
- Assistance to improve independence in medication management should be provided.
- CTT incorporates a pharmacist within its FTE who participates in the clinical review meetings and is available for consumer and clinician consultation/advice.

First on Call (FOC)

Mental health emergencies and crises regarding active consumers for CTT will be responded to promptly and effectively using the Crisis Triage Rating Scale (CTRS) [EMHS Acute Response mental Health Emergencies/Crisis](#)

In the absence of the care coordinator the rostered FOC clinician will provide this response and determine appropriate clinical intervention.

All clinical staff except for medical will participate in the FOC roster. The FOC clinician is expected to keep their rostered shift free of scheduled appointments and remain in the clinic unless required to attend to a FOC duty off site or assist as a second person for contacts considered high risk.

FOC operates from 08.30 – 16.30 managing telephone calls and self-presentations from active consumers when the care coordinator is unavailable.

In the event of planned leave, it is the clinician's responsibility to make alternative arrangements with a colleague to swap shifts. If a rostered staff member is unavailable due to unplanned leave, a senior staff member will liaise with the team to cover the shift. In the event of multiple staff members providing cover for a shift, a clinical handover of completed and outstanding issues will be recorded on the FOC handover sheet and verbally communicated to the clinician taking over the shift.

FOC will attend the AHS daily huddle via MS Teams updating the wider MH service of issues pertaining to CTT/EEP e.g. consumers waitlisted for admission, pending MH

assessments that may require admission/ED attendance. Pertinent information regarding BRAG status, bed flow, discharges etc., will be communicated to the CTT/EEP teams at the daily morning stand up meeting.

FOC will regularly monitor the email inbox throughout the shift and respond to messages according to clinical need and urgency, marking those which have been actioned green in the email box. New referrals will be recorded on the CTT referral sheet on W drive.

When the FOC clinician is called upon to accompany other clinicians on home visits, they must ensure cover is provided to undertake the FOC duties in their absence. If unable to source a member of staff, this must be escalated to the team leader or delegate.

If several category A, B or C CTRS responses are required simultaneously in a care coordinators absence, FOC will liaise with the team for assistance and will prioritise workload accordingly. Escalation to the CTT team leader and treating doctor in these circumstances must occur to ensure responses are actioned as per the CTRS.

Role of Clinic Clerk

- Notifies FoC of unplanned consumer self-presentations if regular Care Coordinator is unavailable.
- Transfers calls to FoC via switchboard if regular Care Coordinator is unavailable and consumer is requiring further clinical advice as required.
- Takes phone messages and sends relevant demographic and context details to FoC via the nominated FoC email when staff unavailable to take calls (not inclusive of urgent calls which are escalated to the next available CTT staff member to take).
- Provides clerical support to FoC in way of:
 - Responding and actioning face-to-face enquiries from FoC.
 - Responding and actioning email requests from FoC.
 - Booking required medical appointments for consumers via patient information systems.
 - Ordering and maintaining required forms and stationery for FoC use.
 - Other clerical duties as directed by Clerical Coordinator in accordance with FoC and service needs.

After-hours Service

The ATT provides an after-hours service from 4.30pm until 10.00pm Monday to Friday and from 8.00am until 10.00pm on weekends and public holidays.

All callers to the OAC after 4.30pm will be advised via a recorded message to call the Mental Health Emergency Response Line (MHERL). MHERL clinicians will contact after hours ATT advising if consumer contact is required.

There is no facility for individuals to self-present to the after-hours ATT at the OAC. This service is provided by the AHS Emergency Department.

The after-hours ATT clinicians provide the following services:

- Telephone triage/advice.
- Emergency community assessments
- After-hours input to active AMHS ATT consumers.
- After-hours input to active AMHS CTT consumers – work deemed unable to have been completed within office hours, not routine business.
- After hours input to AMHS Older Adult consumers.
- Response to Mental Health Police Co Response Team

Referring to the ATT for after-hours input

All referrals to the ATT after-hours clinicians will be via completion of the ATT After-hours Request Form and a verbal (ISOBAR) handover given to the receiving ATT clinician. For referrals to ATT which may result in an inpatient admission, the CTT care coordinators must ensure that all required documentation is updated prior to the referral.

Community Home Visits

- Ensure that community/home visits are conducted in accordance with the EMHS Community/Home Visiting Policy; and
- Armadale Mental Health Service, Community Home Visiting Safety Guideline

Clinical Handover for planned leave

In order to mitigate potential risks and ensure continuity of care when a care coordinator has planned leave of one week or more they will:

- Provide a comprehensive written clinical PSOLIS handover sheet detailing any necessary interventions for consumers and which clinician this has been allocated/handed over to
- The handover sheet will be saved on W drive patient information folder under CTT caseload handover sheet.
- The handover will be filed as CC name and date of leave e.g. A. Person 1.1.21 – 12.1.21
- For consumers who have been allocated an interim clinician for the period of leave a Service event will be entered on PSOLIS identifying 'clinical handover;' and indicating the care coordinator and clinician receiving the information.
- On return from leave, the care coordinator will transfer the handover document on W drive to a sub folder marked Archive to enable handover sheets to remain available for future reference.

Escalation of Care

On recognition that a consumers mental or physical health is deteriorating, it must be escalated to provide the most appropriate and timely intervention. Escalation should occur if the consumer:

- Shows deterioration in mental or physical health.
- Declines treatment and support
- Is at significant risk of injury to themselves or another person.
- Moves out of area/is not locatable.
- Requests an early review/carer concern.

The treating doctor, clinician/MDT will determine the planned intervention which may include but is not limited to:

- Increased contact/frequency of assessment
- Early Medical Review
- Mental Health Act Assessment
- Admission
- CTO management, e.g. breach, order to attend.

Consumers who become No Fixed Abode (NFA) after activation:

There may be circumstances whereby a consumer after activation with CTT does not have fixed address within the catchment area and is homeless. The consumer will not be disadvantaged if they become NFA status. In these circumstances, the treating team will:

- Assist the consumer to attempt to access appropriate and suitable accommodation.
- Make efforts to maintain treatment e.g. LAI if the environment is safe to do so
- Make concerted efforts to maintain contact with the consumer to monitor and assess the consumers wellbeing.
- If a consumer becomes uncontactable, the disengagement process will be followed.
- Psolis alerts will be utilised to communicate any delays in treatment or risk issues should the consumer present to another service.
- Each case will be discussed within the treating team so that all risk issues and last MSE can be considered to inform clinical decision making for consumers of concern.

Essential Care Model

At times of critical workforce deficits which impact upon service delivery, with the support and approval of the Medical and Service Directors, CTT may be required to prioritise essential and urgent care to consumers. During these times referrals to CTT may be deferred and service delivery provided by ATT extending their program to 15 weeks. This change of care is intended to be temporary and reviewed regularly by the Program Manager and Team Leaders within the Adult Community Teams.

Prioritisation includes:

- Crisis intervention
- Consumers waitlisted for in patient admission.

- Consumers on CTO's
- Consumers prescribed Clozapine Therapy.
- Consumers who have not had clinical contact within the last 4 weeks.

Supervision and Line Management

CTT clinicians will be provided line management from a senior member of staff. Line supervision is provided monthly on operational matters including but not limited to, performance, workload management and human resource related matters.

The following processes fall within the scope of line managers responsibilities.

- Peak Performance Review; required to be undertaken annually in accordance with the EMHS Peak Performance Policy
- Performance Management; as outlined in the WA Health managing Unsatisfactory and Substandard Performance Policy Framework,
- Fitness to work; as outline in the EMHS Fitness for Work policy

In house training/education will reflect best practice that meets local demands, challenges, and needs, with consideration given to further improving skills and knowledge in teamwork, case formulation, communication, documentation, electronic record keeping and the model of service. CTT staff are required to attend a mandatory skills assessment (MSA training day annually).

Clinical Supervision can be accessed to support clinicians in their professional environment with the goal of improving clinical practice and consumer outcomes.

Please refer to the following: [EMHS Clinical Supervision in MH Policy](#). There is a requirement for the staff member to complete a supervision contract agreement outlining the agreed goals, objectives, responsibilities, format, method, and model for clinical supervision.

Employment Support

Individual Placement and Support (IPS) is an evidenced-based integrated approach to mental health clinical intervention and employment support.

Armadale Mental Health Service (AHS) is a key stakeholder with IPS employment specialists Matchworks in integrating employment and vocation services with clinical mental health, focussing on the individual needs of consumers diagnosed with severe and enduring mental health issues who are seeking to enter or remain in education or employment.

Members of the MDT can refer consumers to the program for assistance and support in exploring vocational options and achieving employment. [Matchworks | Job Seeker and Disability Employment Service Provider](#)

Team Meetings

Daily Huddle

Occurs via MS Teams at 8.30 which includes ED, Consultation Liaison, In patient wards, Crisis and Home Treatment Team, (CHRTT), and all AHS community mental health teams.

The meeting includes patient flow between all areas of the mental health service and is attended by the rostered CTT FOC clinician and a senior member of the team.

Stand Up Meeting

May occur each weekday 8.45:00 and provided the forum to discuss the day's business, Risk, consumers of concern, overdue depot's and staffing needs. All discussions pertaining to consumer care needs to be recorded in DMR

Intake and Allocation Meetings

Intake Meetings within Community Mental Health occur daily (ATT), three times a week (CTT) and Weekly (EEP) with members of the MDT present. The Teams will review the referral information contacting the referrer if insufficient details are available. A CTT representative may attend ATT intake meetings to discuss referrals suitable for longer term management.

The team will consider possible treatment requirements including assessment, medication, psychosocial needs, level of acuity and discipline specific requirements in identifying the most appropriate community program to meet the consumers current needs.

When allocating the consumer a care coordinator, clinician suitability including caseload acuity, skills and experience are considered to enable a best fit approach as per the [EMHS Care Coordination in MH Services Policy](#)

Business Meeting

Held on the third Thursday of every month, at 3pm. This meeting is to discuss CTT operational matters, KPI's, policies, issues, staffing, policy/guidelines changes work and safety, facilities, etc. Staffs are invited to submit agenda items for discussion each month and are expected to attend.

Catchment Area:

CTT operates within the following catchment areas:

Armadale	Darling Down	Kelmscott	Oakford
Ashendon	Forrestdale	Kenwick	Oldbury
Beckenham	Gosnells	Keysbrook	Orange Grove
Bedfordale	Harrisdale	Langford	Piara Waters
Brookdale	Haynes	Lesley	Roleystone
Byford	Hilbert	Maddington	Serpentine
Camillo	Hopeland	Mardella	Seville Grove
Canning Mills	Huntingdale	Martin	Southern River

Canning Vale	Jarrahdale	Mount Nasura	Thornlie
Cardup	Karragullen	Mount Richon	Wungong
Champion Lakes	Karrakup	Mundijong	

Abbreviations and Acronyms

AHS	Armadale Health Service
AKG	Armadale Kalamunda Group
AMHS	Armadale Mental Health Service
ASSIST	Alcohol, Smoking, Substance Involvement Screening Tool
ATT	Assessment and Treatment Team
CTT	Clinical Treatment Team
DMO	Duty Medical Officer
DNA	Did not attend
DOB	Date of Birth
DSM	Diagnostic and Statistical Manual of Mental Disorders
DST	Discharge and Support Team
ED	Emergency Department
EMHS	East Metropolitan Health Service
ext.	Extension
FOC	First on Call
GP	General Practitioner
GPs	General Practitioners
ICD	International Classification of Diseases
iSoBAR	Identify, situation, observations, background, agree plan, read back
LUNSERS	Liverpool University Neuroleptic Side Effect Rating Scale
MDT	Multi-Disciplinary Team
MHAF	Mental Health Assessment Form
MHLN	Mental Health Liaison Nurse
NOCC	National Outcomes and Casemix Collection
NUM	Nurse Unit Manager
EDMHLN	Emergency Department Mental Health Liaison Nurse
PSOLIS	Psychiatric Services Online Information System
PSP	Personal Support Person

RAMP	Risk Assessment and Management Plan
TSDP	Treatment Support and Discharge Plan
TRS	Triage Rating Scale
WebPAS	Web based Patient Administration System

Bibliography

1. Chief Psychiatrist of Western Australia
 - [Chief Psychiatrist's Standards for Clinical Care](#)
2. Department of Health Operational Directive / Policy
 - [Enterprise Bed Management - Mental Health Business Rules](#)
 - [WA Health MP 0095 Clinical Handover Policy](#)
 - [Mental health Consumer Medication Information MP0069/17](#)
 - [Triage to Discharge – Mental Health Framework for State-wide Standardised Clinical Documentation](#)
3. East Metropolitan Health Service Policy / Framework
 - [Care Coordination in Mental Health](#)
 - [EMHS Community/ home visiting Policy](#)
 - [EMHS Patient Flow and Bed Capacity Framework](#)
 - [EMHS Clinical Supervision in Mental Health Policy](#)
4. Armadale Mental Health Service Policy / Guideline
 - [Admission of Patients from Community Mental Health Service to Armadale Mental Health Inpatient Units](#)
 - [Assessment and Admission Documentation Requirements](#)
 - [Assessment Treatment Team Process Manual](#)
 - [Community Home Visiting Safety](#)
 - [LUNSERS \(Liverpool University Neuroleptic Side Effect Rating Scale\) - using for monitoring Neuroleptic side](#)
 - [Metabolic Screening](#)

Appendix A: Mental Health Triage Rating Scale – EMHS Community Services

Code/ description	Response type/ time to face-to-face contact	Typical presentations	MH Service action/ response	Additional actions to be considered
A Current actions endangering self or others	Emergency services response IMMEDIATE REFERRAL	- Overdose - Other medical emergency - Siege - Suicide attempt / serious self harm in progress - Violence / threats of violence and possession of weapon	Community clinician* notify ambulance, police and / or fire brigade. Contact fellow clinicians	Keeping caller on line until emergency services arrive. Ascertain what assistance is required. Notification of other relevant services (e.g. child protection)
B Very high risk of imminent harm to self or others	Very urgent MH response WITHIN 2 HOURS	- Acute suicidal ideation or risk of harm to others with a clear plan and means and / or history of self harm or aggression - Very high risk behaviour associated with perceptual / thought disturbance, delirium, dementia or impaired impulse control - Urgent assessment requested by Police	Face to face assessment required AND / OR Community clinician advice to attend a hospital emergency department when community clinician cannot attend in timeframe or ED treatment required	Providing or arranging support for consumer and / or carer while awaiting face-to-face MHS response (e.g. telephone support/therapy; alternative provider response). Telephone secondary consultation to other service provider while awaiting face-to-face MHS response. Advise caller to ring back if the situation changes. Arrange parental/carer supervision for a child/adolescent, where appropriate.
C High risk of harm to self or others and/or high distress, especially in absence of capable supports	Urgent MH response WITHIN 12 HOURS	- Suicidal ideation with no plan and/or history of suicidal ideation - Rapidly increasing symptoms of psychosis and/or severe mood disorder - High risk behaviour associated with perceptual/thought disturbance, delirium, dementia, or impaired impulse control - Unable to care for self or dependants or perform activities of daily living - Known consumer requiring urgent intervention to prevent or contain relapse	Community clinician face-to-face assessment within 8 hours AND Community clinician telephone follow-up within ONE HOUR of triage contact.	As above Obtaining corroborating/additional information from relevant others.
D Moderate risk of harm and/or significant distress	Semi-urgent MH response WITHIN 48 HOURS	- Significant client/carer distress associated with serious mental health illness (including mood/anxiety disorder) but not suicidal - Early symptoms of psychosis - Requires priority face-to-face assessment in order to clarify diagnostic status - Known consumer requiring priority treatment or review.	Community clinician face-to-face assessment	As above
E Low risk of harm in short term or moderate risk with high support/stabilising factors.	Non-urgent MH response Within 2 weeks	- Requires specialist mental health assessment but is stable and at low risk of harm in waiting period - Other service providers able to manage the person until MHS appointment (with or without MHS phone support) - Known consumer requiring non-urgent review, treatment or follow-up	Continuing care or equivalent. Face-to-face assessment	As above
F Referral: not requiring face-to-face response from MHS in this instance	Requires further triage contact/ follow up	- Other service (e.g. GPs, private mental health practitioners) more appropriate to person's current needs - Symptoms of mild to moderate depressive, anxiety,	Community clinician to provide formal or informal referral to an alternative service provider or advice to attend a particular	Facilitating appointment with alternative provider (subject to consent/privacy requirements), especially if alternative intervention is time critical

Based on Victorian Government Triage MH rating scale and NSW MH Rating Scale* Community clinician- any clinician (medical, nursing, allied health) within Assessment and Treatment Team (ATT) and/ or other MH community teams

Appendix B: AHS CTT Hostel In-Reach Process Manual

Adult Community Mental Health Clinical Treatment Team Hostel In-Reach Process Manual

Clinical Treatment Team

Hostel In-Reach

Authorisation

Compiled By:	Program Manager Adult Community Mental Health	Executive Sponsor:	Service Director Mental Health
Date First Issued:	27/08/2025	Consumer Group/Committee Consulted (if applicable):	
		Endorsed By:	AKG - Mental Health Service Clinical Governance Committee
Revised By:		Executive Sponsor:	

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About this Manual

This manual provides a framework of processes for the Armadale Mental Health Service (AMHS) Clinical Treatment Team (CTT) Hostel In-Reach (HIR) specific to the HIR role. This manual is to be read in conjunction with the overarching CTT process Manual which provides more detailed information on, Access, roles, responsibilities, and process. The HIR clinicians are part of the wider Adult Community Mental Health Service based at the Orchard Avenue Centre. [CTT Process Manual](#)

Model of Care

The ACMHS, CTT model of service describes the function of one component of a broader mental health system.

HIR is a portfolio within CTT which provides medium to long term recovery focussed, specialist community mental health care services for individuals living in Licenced Psychiatric Hostel accommodation within the AMHS catchment area.

Key Functions

A key focus for the HIR clinicians is to encourage and facilitate engagement to improve the mental and physical health outcomes of consumers within Licenced Psychiatric Hostels.

This service is provided with a recovery orientated approach that is person centred, strengths based, collaborative and empowering. The lived experience and insights of people with mental health issues and their families/carers are at the heart of recovery orientated culture. The service meets the National Standards (NSQHS) for mental health services and the National Practice standards for the mental health workforce and will be audited and reviewed alongside the broader mental health systems. CTT HIR operates on the premise that people can and do recover from mental illness. The team may broker the skills and expertise of other multi-disciplinary team members to meet the identified needs of the consumer e.g. social work, psychology, occupational therapy.

The EMHS and Armadale Mental Health Adult Program principles are defined within the CTT manual.

The HIR Clinicians will be available between the hours of 8:30-4:30pm Monday to Friday excluding public holidays. Mental health support outside these hours will be undertaken as per existing processes outlined in the respective service agreements.

The **Key Functions** are:

1. Provision of assertive clinical management to minimise the impact of mental illness on consumers and their family and/or nominated personal support persons (PSP).
2. Provide a consistent link between the psychiatric hostel and AMHS.
3. To provide comprehensive individualised assessment of the consumer to ensure holistic care planning.
4. Facilitate engagement with physical health screening, monitoring, and access to primary care physicians.
5. To monitor the ongoing physical health of consumers unable to access primary care. Facilitation of access to a broad range of clinical and non-clinical services to assist recovery.
6. To advocate for equity in access to physical health care.
7. Provide a standardised approach to physical health care and in particular metabolic screening.
8. Working in partnership with consumers to develop their personal support systems and to assist them to engage in their communities.
9. Engagement with non-government organisations, primary care, and other service providers to assist access to services across the recovery continuum and ensure smooth transitions of care.
10. Supporting hostel staff, families and nominated PSP to access any appropriate support.

11. Increase the level of mental health literacy and understanding to promote social inclusion and health promotion.

Access Criteria

Consumers will meet the CTT entry criteria for continuing mental health care and treatment. They will:

- Be an active client of AMHS.
- Have a current residential address at one of the Licenced Psychiatric Hostels within the ACMHS catchment.

Consumers referred to the service may also be seen by the Assessment and Treatment Team (ATT) or the Transition Team for assessment and short-term interventions if appropriate.

Licenced Psychiatric Hostels:

BP Luxury Care: Roshan Group - Maddington

Specialised licensed MH facility for 44 people with serious and persistent mental illness who are at risk of homelessness and who require support to enable them to live in the community.

The support required is determined on an individual basis and includes individual and group support and can include assistance with activities of daily living, and medication prompts. ADL activities, medication supervision and prompts.

Richmind Community Options – Hicks Road - Kelmscott

The community options program provides home-like supported accommodation for 8 adults with a severe and persistent mental illness with high support needs. The accommodation provided is split into two self-contained single sex residential houses.

The aim of the organisation is to maintain and/or improve residents' quality of life and support services that address whole life needs.

As part of Richmind's transition to the National Disability Insurance Scheme (NDIS) and changes in state funding long term residential services provided by the organisation will end in 2026.

St. Bartholomew's House Recovery Village – Arnott Villas - Kelmscott

Supported accommodation for up to twenty-two residents with serious and persistent mental health issues and who are at risk of homelessness.

The service is based on the principles of recovery and provides low to medium level support to have a meaningful life in the community. A maximum of 4 hours daily support is provided. Accommodation is provided in 2-bedroom shared living units.

AMHS has either a Memorandum of Understanding (MOU) or Service Level Agreement (SLA) in place with each of the hostels.

These agreements between services support ACMHS and its partners in working collaboratively and in support of the individual organisational intent.

The aim of the collaboration is to provide effective interventions for consumers and wrap around supports.

MOU/SLA's outline the roles and responsibilities of each agency in relation to delivery, access, management, and evaluation of services.

The principles of the agreements are:

- Person focused.
- Recovery focused.
- Partnership of responsible agencies
- Mutual respect and communication
- Continuing liaison

The hostels have facilities to home a combined total of 73 residents not all of whom are active with AHS Community Mental Health Services.

HIR Clinician Roles and Responsibilities

- Provide care coordination for each active consumer of Armada Mental Health Service (AMHS)
- Provide clinical intervention based on the consumers MH needs.
- Engage and monitor physical health supporting active treatment in primary care setting as appropriate.
- To visit the consumers as frequently as agreed in consultation with the hostel provider.
- Liaise with key stakeholders to develop a TSDP in collaboration with recovery goals.
- Provide updated Treatment Support and Discharge Plan and Risk Assessment Management Plan to the respective hostel.
- Medical oversight is currently provided by the medical team within CTT.

Care Coordination

ACMHS follows the EMHS Care Coordination in Mental Health Services Policy. Please review this document to identify your roles and responsibilities within this Policy. [EMHS Care Coordination in Mental Health Services](#)

Collaboration and Consultancy

All hostels have an individual referral process. The Team Leader and Hostel in-reach CNS can provide consultancy and information of pending referrals in relation to known and potential risk factors.

The HIR clinicians provide support only for consumers active with Armadale CTT. They can offer pathways for assistance and general advice to other residents or staff, but it cannot provide clinical interventions to non-active residents.

HIR clinicians will provide communication to general practitioners and specialist clinicians involved in the care of consumers.

Confidentiality and Information Sharing.

It is acknowledged that all providers are bound by legal and ethical obligations in relation to confidentiality and information sharing. This includes the Privacy Act (1988) and MHA (2014).

Consent forms to release and share information are completed and signed by the consumer where the sharing of information may enhance services being received. Both ACMHS and the hostel will seek written permission on entry to the respective services.

Mental Health Advocacy.

People who are residents of a private psychiatric hostel are entitled to request assistance from a Mental Health Advocate (MHA). HIR clinicians will support and assist consumers to contact the MHA if required. Hostel residents rights are further explained on the Government of Western Australia website [WA Government - Psychiatric Hostels](#)

Assessment

In addition to mental state and risk assessments, HIR clinicians will undertake a comprehensive and holistic assessment of the consumers' needs based on the psychosocial model, with the key pillars of biological, psychological and social factors being considered to assist in the formulation of the Treatment Support and Discharge Plan.

Sexual Health and Safety

Everyone accessing mental health services has the right to feel and be psychologically and physically safe. This includes being free of, and feeling safe from, behaviour of a sexual nature that is unwanted, or makes a person feel uncomfortable, afraid, or unsafe.

It is important to note that some consumers accessing our service may be more vulnerable.

- Consumers with a cognitive impairment (including cognitive impairment for mental illness) are vulnerable to abuse.
- Individuals who have a history of physical and sexual abuse can have a higher degree of lifetime exposure to sexual abuse and violence.
- LGBTQI+ consumers experience increased vulnerability to sexual victimisation.

As part of a comprehensive assessment, consumers are asked how they identify in relation to gender, ensuring gender sensitive approaches and staff gender preference. Clinician allocation will consider the consumers preferences alongside clinical need and staff availability.

Sexual health will form part of the comprehensive assessment, including sexual behaviour/activity, sexual difficulties, contraception, HIV and STI's with appropriate screening being encouraged/offered.

Sexual Safety of Consumers of Mental Health Services

Physical Health Assessment

Physical and oral health will be routinely assessed and documented. This may be conducted by a health service provider external to the CTT but will be considered as part of CTT assessment. Physical and oral health issues will be routinely addressed in partnership with all consumers of CTT.

- All consumers will be supported to engage with a nominated GP.
- Where identified barriers exist e.g. language, gender, the consumer will be supported in identifying a GP who is able to meet their needs and preferences.
- The treating doctor and Care Coordinators will maintain written communication with the GP throughout care and any changes in treatment communicated.
- The GP will be contacted for collateral and will be provided with results/ongoing care recommendations.
- Physical health care will encompass.
 - Health Care Assessment – Prevention and Health Promotion
 - Age-appropriate screening e.g. breast, prostate, skin
 - Smoking Cessation
 - Drug and Alcohol
 - Metabolic Screening
 - Dental
 - Diet and Exercise
 - Medication
 - Access and intervention
 - Integrated care
- Treatment support and discharge plans will specifically address physical health care needs.

Drug and Alcohol Assessment

Co-occurring mental health and alcohol and other drugs (AOD) needs are highly

prevalent in the CTT cohort of consumers. Assessments of alcohol and drug use should be undertaken on entry to CTT using the ASSIST form. CTT is expected to complete these assessments unless already undertaken and reviewed whenever there is a change in AOD consumption/use. Management of AOD issues should occur as part of the treatment and clinical review process. Additional Screening may be undertaken utilising:

- Alcohol Use Disorders Identification Test Screening Instrument (AUDIT).
- Severity of Alcohol Dependence Questionnaire (SADQ-C)
- Drug Abuse Screening Test (DAST)
- Severity of Dependence Scales (SDS)

Clinical Review

Clinical reviews are a formalised meeting with the multi-disciplinary team to review identified management actions and National outcomes and Casemix Collection (NOCC) data. The review meeting participants share information to ensure consumers receive ongoing assessment, monitoring, and treatment via a coordinated multi-agency response.

A clinical review will be conducted every 91 days The reviews are scheduled every week and chaired by the consultant psychiatrist.

The process is outlined in the CTT Process Manual.

Treatment Support and Discharge Planning

Involvement of family and significant others/nominated PSP's and other service providers is recognised as integral to consumer engagement and is strongly linked to better clinical outcomes.

TSDPs are formulated on entry to CTT and are formally reviewed every three months and updated at a frequency determined by change or need. Plans will be developed in collaboration with all relevant providers, in the presence of the consumer and their identified support network and will:

- Be recovery focused and unique to the individual.
- Detail specific interventions,
- Identify clear treatment and discharge goals in accordance with the clinically significant needs identified from the collection of NOCC data and psychosocial assessment.
- Include the specific issue (including physical) and what is being done by who, when and where.
- Be developed in partnership with the consumer, PSPs and other agencies as defined by the consumer's goals.
- Reflect and support additional goals identified with hostel provided
- Be printed and offered to the consumer with a signed copy uploaded to DMR.
- Shared with the hostel provider.

As the consumer transitions through their recovery stages the level and type of support will be determined by the consumer's goals. Every effort must be made to supply resources to families/ carers/significant others which may be helpful to them.

Transition to Primary Care

Discharge can be a time-intensive aspect of care coordination. The responsibilities of CTT in discharge planning can be summarised into key areas. These areas also overlap with the criteria mentioned in the National Safety and Quality Health Service (NSQHS) Standards [NSQHS](#) and the Chief Psychiatrist's Standards for Clinical Care [OCP Clinical Care Standards](#)

- Engage in collaborative discharge planning at an early stage of treatment with the consumer and their family/nominated PSP and key stakeholders.
- Safe transition of care – the nature and length of the transition period will vary according to the history and care needs of the consumer. This may include:
 - Outright transfer of care to GP.
 - Transfer of care to service outside of catchment
 - Transfer from HIR stream to CTT if the consumers leave the hostel provider but remains within catchment and requires ongoing mental health services.
- Provide written and verbal treatment-related information for the consumer being discharged to continuation of treatment in an alternative setting.
- Re-entry pathways – If the consumers' needs change post discharge and their existing plan becomes inappropriate to their needs, they have clear advice on where to seek support, treatment, or re-referral to the specialized service.

[AKG Scheduling, Rescheduling and DNA of Appointments in Community mental Health Clinics](#)

Clozapine and Depot Clinics

The Clozapine and Depot clinics are facilitated by a Registered Nurses (RN's). Consumers resident in licensed hostels will be encouraged to attend these clinics for treatment and review.

Consumers on Clozapine attend the clinic to ensure their mental and physical health is monitored as per the [AKG Clozapine Prescribing, Administering and monitoring Procedure](#)

Consumers who attend the clinic for administration of depot medication will have their mental and physical health assessed, any identified concerns are to be communicated to the Care Coordinator and treating doctor prior to the medication administration.

Alternatively, depot administration may be undertaken by the HIR clinician on the hostel premises where there is a clearly identified clinical need

Depot medications are administered in line with the [AKG Long Acting Injectable Depot Medication in Community Mental health Clinics Procedure](#)

Outcomes and Key Performance Indicators (KPI)ACMHS is required to report outcomes for consumers in hostels to the Mental Health Commission on a 6 monthly basis. Shown below is expected reporting KPI's and measurements.

Armada Mental Health Service

Outcomes	Outcomes	Definitions
	Improvement in mental health outcomes: - Number of consumers who demonstrated stabilisation or improvement in mental health outcomes	Numerator = number of consumers treated with an improvement and/or stabilisation in HoNOS scores during the reporting period. Denominator = total number of consumers treated with at least 2 HoNOS scores (activation; to most recent review and/or discharge). Numerator = number of ICLS participants treated with an improvement and/or stabilisation in NOCC* scores during the reporting period. Denominator = total number of consumers treated with at least 2 NOCC scores (admission, most recent review and/or discharge).** *K10/K10+ instrument **Review/discharge score to have been measured during the reporting period
	Reduce unplanned readmissions to acute specialised mental health inpatient services: - Number of consumers who have unplanned readmission to an acute specialised mental health inpatient unit.	Numerator = number of overnight separations of consumers from acute specialised mental health inpatient unit, that are followed by a readmission to the same or another acute specialised mental health inpatient unit within 28 days. Denominator = number of overnight separations of consumers from acute specialised mental health inpatient units.

KPIs	KPIs	Measures
	Service Contacts	Number of service contacts Number of service activation Number of consumer reactivations Number of consumer deactivations
	Providing referrals to other support services as appropriate* * onwards referral on discharge from the service Definition: [Referred on] following deactivation from program within reporting period	Facilitated referrals to other services Number of referrals made to other services Referral destination (i.e. service provider)
KPIs	Reviews completed	Number of psychiatric reviews (every 91 days)* *Clinical reviews Number of risk and safety reviews (every 3 months)* *total count of number of RAMPs within reporting period Number of medication reviews (every 6 months)* *total number of service events count of "assessment" + count of "medication review" within the 6 month period
	Provision of individualised planning, including shared identification of recovery goals, and care coordination/ Provision of integrated/joint consumer care through collaboration between consumer, HSP and NGO.	Number of physical health assessments (every 12 months in collaboration with GP)
	Service level agreement	90% of consumers that have signed a PSOLIS treatment, support and discharge plan/ management plan developed in consultation with consumer, Hostel and other stakeholders. Current Service Level Agreement in place between Hostel and HSP.

Glossary

ATT	Assessment and Treatment Team
ASSIST	Alcohol, Smoking, Substance Involvement Screening Tool
AUDIT	Alcohol Use Disorders Identification Test Screening Instrument
DAST	Drug Abuse Screening Test
EMHS	East Metropolitan Health Service
FOC	First on Call
GP	General Practitioner
GPs	General Practitioners
KPI	Key Performance Indicator
NOCC	National Outcomes and Casemix Collection
PSOLIS	Psychiatric Services Online Information System
PSP	Personal Support Person
RAMP	Risk Assessment and Management Plan
SADQ-C	Severity of Alcohol Dependence Questionnaire
SDS	Severity of Dependence Scales.
TSDP	Treatment Support and Discharge Plan
UWA	University of Western Australia

Bibliography

1. Chief Psychiatrist of Western Australia

- [Chief Psychiatrist's Standards for Clinical Care](#)

2. Department of Health Operational Directive / Policy

- [WA Health MP 0095 Clinical Handover Policy](#)
- [Triage to Discharge – Mental Health Framework for State-wide Standardised Clinical Documentation](#)

3. East Metropolitan Health Service Policy / Framework

- [Care Coordination in Mental Health](#)

4. Armadale Mental Health Service Policy / Guideline

- [Assessment and Admission Documentation Requirements](#)

- [LUNTERS \(Liverpool University Neuroleptic Side Effect Rating Scale\) - using for monitoring Neuroleptic side](#)
- [Metabolic Screening](#)

5. Government of Western Australia Mental Health Commission/UWA

- [Clinical Guidelines for the Physical Care of mental Health Consumers](#)

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